

AFib Anticoagulation Restart Timing After Acute Ischemic Stroke

CLINICAL EFFICACY & SAFETY

RCT and individual-patient meta-analysis data support early DOAC initiation in carefully selected AFib-related ischemic stroke patients, especially mild-to-moderate infarcts without high-risk hemorrhagic transformation. Early treatment has not shown excess symptomatic intracranial hemorrhage (sICH) versus delayed treatment and may reduce recurrent ischemic stroke; DOACs are preferred over warfarin for most nonvalvular AF patients when anticoagulation is indicated.

1. Stroke Severity Classification (ELAN Imaging Criteria)

Minor / Small Infarct

- Ischemic infarct ≤ 1.5 cm on brain imaging; ELAN did not enroll tissue-negative TIA. NIHSS can guide bedside risk but is not the ELAN definition.

Moderate Infarct

- Cortical superficial-branch lesion, internal border-zone lesion, or deep-branch lesion > 1.5 cm.

Major / Large Infarct

- Complete vascular territory, ≥ 2 moderate lesions, large multilobar infarct, or brainstem/cerebellar lesion > 1.5 cm; apply the protocol's multiple-lesion rules.

Key clinical boundaries

There is no validated combined day-by-severity schedule or mandatory day-specific rescan rule in this card. ELAN compared early treatment within 48 hours for minor/moderate infarcts and on days 6–7 for major infarcts with later treatment on days 3–4, 6–7 and 12–14, respectively. OPTIMAS compared initiation within 4 days with days 7–14. CATALYST pooled early (≤ 4 days) versus later (≥ 5 days) strategies. Use each trial's eligibility and hemorrhagic-transformation exclusions, the evolving clinical and imaging findings, and shared clinical judgment; severe hemorrhagic transformation is not assigned a start day here.

2. Bedside DOAC Dosing & Adjustment Guide

Drug	Standard Dose	Dose Reduction Criteria
Apixaban (Eliquis)	5 mg BID	Reduce to 2.5 mg BID if ≥ 2 criteria are met: • Age ≥ 80 years • Weight ≤ 60 kg • Serum creatinine ≥ 1.5 mg/dL
Rivaroxaban (Xarelto)	20 mg daily with the evening meal if CrCl >50 mL/min	15 mg daily with the evening meal if CrCl is 15–50 mL/min. CrCl <15 or dialysis: specialist assessment; no automatic dose or hold rule here. NVAf dialysis labeling relies on pharmacokinetics; equivalent stroke prevention and bleeding outcomes are unknown.
Dabigatran (Pradaxa)	150 mg BID	Reduce to 75 mg BID if CrCl is 15–30 mL/min. Avoid if CrCl < 15 mL/min.
Edoxaban (Savaysa)	60 mg daily	Reduce to 30 mg daily if CrCl is 15–50 mL/min (US NVAf label). Avoid if CrCl >95 or <15 mL/min. Weight ≤60 kg alone is a US VTE, not NVAf, dose-reduction criterion.

ELAN Trial: Fischer U et al. *N Engl J Med.* 2023;388:2411-2421. **PMID: 37222476** (<https://pubmed.ncbi.nlm.nih.gov/37222476/>)

CATALYST Meta-Analysis: Dehbi HM et al. *Lancet* 2025; **PMID: 40570866** (<https://pubmed.ncbi.nlm.nih.gov/40570866/>). Pooled data (n=5,441) from **ELAN (PMID: 37222476)** (<https://pubmed.ncbi.nlm.nih.gov/37222476/>), **OPTIMAS (Lancet 2024; PMID: 39491870)** (<https://pubmed.ncbi.nlm.nih.gov/39491870/>), **TIMING (Circulation 2022; PMID: 36065821)** (<https://pubmed.ncbi.nlm.nih.gov/36065821/>), and **START (JAMA Neurol 2025; PMID: 40163159)** (<https://pubmed.ncbi.nlm.nih.gov/40163159/>). Early DOAC (≤4 days) vs later (≥5 days) initiation showed no excess sICH (0.4% vs 0.4%) and fewer recurrent ischemic events in pooled data.

US dosing labels: **Xarelto §§2.1, 8.6 (NVAf)** (<https://www.janssenlabels.com/package-insert/product-monograph/prescribing-information/XARELTO-pi.pdf>); **Savaysa §2.1 (NVAf)** (<https://dailymed.nlm.nih.gov/dailymed/lookup.cfm?setid=e77d3400-56ad-11e3-949a-0800200c9a66>); check each current label for interactions and renal dosing.

AFib Guidelines: Joglar JA et al. 2023 ACC/AHA/ACCP/HRS Guideline. *Circulation.* 2024;149:e1-e156. **PMID: 38033089** (<https://pubmed.ncbi.nlm.nih.gov/38033089/>)